
BRIGHTKIDCO EMAIL ECOSYSTEM
AVATAR: AUTISTIC PARENT
DOC 3 OF 6 — LEVEL 2 PROFILE
(Mixed verbal, moderate support needs, 5-7 years)
VERSION 2.0 — END-LEVEL FOR MARKETER WORKFLOW

This doc is a sub-profile within the Autistic Parent avatar.
Read DOC 1 (Master Foundation) first.

Audience share: ~40% of the Autistic Parent audience
(LARGEST SEGMENT).

Selection: Pop-up option "Sensory issues, tried many methods"
→ Tag: avatar_level2

IMPORTANT: This is the MOST COMMON audience and at the same
time the TECHNICALLY HARDEST to convert — high skepticism,
fragile hope, lots of prior therapy experience.

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PART A: WHO IS THE LEVEL 2 MOM

1. DETAILED AVATAR PORTRAIT

THE LEVEL 2 MOM IN PORTRAIT:

Imagine Lisa. She's 38, lives in Phoenix. Her son Ethan is 6, diagnosed Level 2 ASD. He's verbal, but often uses scripts and echolalia. His OT says he has moderate sensory processing differences. His BCBA has been working with him since he was 3 — 20 hours of ABA per week.

Lisa has reduced her marketing career to 25 hours. She balances therapy appointments, IEP meetings, BCBA data sheets, and a second child (Mia, 3, neurotypical). Mia has been potty-trained for 6 months. Ethan has been in pull-ups since birth.

Lisa has tried in the last 3 years:

- Oh Crap Method (stopped Day 2)
- Sticker charts (Ethan was bored)
- Social stories with Velcro images (no effect)
- Picture Communication Cards (used for other things, not potty)
- BCBA protocol with scheduled sits (he sits, doesn't go)
- 4 different potty seats (sensory issues with all)
- OT-recommended weighted blanket pre-toilet (no success)
- Reward charts with special interests (short success, then regression)

Ethan can sit on the toilet. He is prompt-dependent: goes when brought, never initiates. Holds bowel movements until bedtime pull-up. Sensory issues: the flushing sound triggers meltdowns.

Lisa has had OT consultation: "interoception might be the problem." She bought Kelly Mahler's book. Read it halfway. Understands the concept but doesn't know what to DO specifically.

She's tired. She's not hopeless — but when someone says "easy solution", she scrolls away.

At BrightKidCo, she is skeptical AND interested

simultaneously.

THAT IS THE LEVEL 2 MOM.

CORE CHARACTERISTICS:

- Has HOPE WITH CRACKS (intact but wounded)
 - Skepticism is default mode
 - "I've tried everything" is TRAUMA RESPONSE, not just statement
 - Has deep experience with therapies
 - Trusts science, NOT marketing
 - Reacts allergically to "miracle product" language
 - Ready to invest if mechanism convinces
 - Has accepted her identity as "autism mom"
-

2. DEMOGRAPHICS & CONTEXT

AGE & FAMILY:

- Mom: 30–42 years
- Second child often already there (often neurotypical)
- Comparison pain is real
- 15-20% are single moms

GEO DISTRIBUTION:

- USA: 65%
- UK: 20%
- AU/NZ: 10%
- Canada: 5%

INCOME LEVEL:

- Middle income dominant
- Lower-middle income: ~30%
- Medicaid usage: ~25%
- Career often reduced for caregiving

EMPLOYMENT STATUS:

- 45% working (often part-time)
- 45% stay-at-home/primary caregiver
- 10% career-focused with high childcare investment

- College-educated: ~75% (often very informed)

CHILD COMORBIDITIES (typical Level 2):

- Sensory Processing Disorder: ~95% (moderate to severe)
- ADHD: ~40%
- Anxiety: ~35%
- Speech delays (often in therapy): ~70%
- GI issues (chronic): ~50%
- Sleep disorders: ~45%

SCHOOL STATUS:

- Mainstream with 1:1 aide: ~40%
- Special needs preschool/school: ~50%
- Mainstream without support: ~10%
- IEP (Individualized Education Plan): ~85%

DIAGNOSIS STATUS:

- ~85% formally diagnosed Level 2
- ~10% Level 1 diagnosis but functions like Level 2
- ~5% in re-evaluation

THERAPY SETUP (typical):

- BCBA + ABA: 20-30 hours/week
- OT: 1-2x/week
- Speech: 1-2x/week
- Sometimes: Physical Therapy, Music Therapy

3. THE LEVEL 2 CHILD IN DETAIL

AGE:

- 5-7 years dominant
- Sometimes younger 4, sometimes older 8

VERBAL PROFILE:

- Mixed verbal
- Often speaks in scripts (movies, books, routines)
- Echolalia common
- Sometimes AAC device as backup
- Can express needs on good days
- On bad days: minimal verbal

- "Functional language" is not the same as "communicative language"

COGNITIVE:

- Highly variable — some academically advanced, some significantly delayed
- Special interests very deep
- Routine-oriented intensely
- Can do structured learning
- Often has limited generalization (can do it with BCBA, not at home)

SENSORY:

- MODERATE TO SEVERE sensory differences
- Often hyper-sensitive (sounds, light, textures, smells)
- Hypo-sensitive for interoception (FEELS LITTLE body signals)
- Sensory overload triggers crisis
- Toilet environment is often trigger (flush, light, echo)

BEHAVIOR:

- Meltdowns regularly (1-3x/week typical)
- Massive routine sensitivity
- Special interests dominant
- Social interaction limited or specific
- Self-stimulatory behaviors (stimming) typical

POTTY TRAINING PROFILE — LEVEL 2 SPECIFIC:

THE TYPICAL LEVEL 2 POTTY SITUATION:

The child:

- CAN sit on the toilet (with acclimation)
- HAS developed prompt-dependent skills
- INITIATES rarely or never themselves
- HOLDS bowel movements until pull-up returns (withholding)
- HAS sensory triggers in the bathroom
- SHOWS sometimes partial successes
- BUT: never consistent over more than days
- BUT: school trained ≠ home trained

THE LEVEL 2 PARADOX (see Section 5):

"She goes for the teacher, but not for me."

TYPICAL LEVEL 2 ACCIDENTS:

- Holds for 8 hours at BCBA, goes seconds after coming home
- Sits 20 minutes without effect, then accident 5 min later
- Has 3 weeks of success, then 2 months of regression
- Goes at school, refuses at home
- Sensory meltdown before toilet stops attempt

4. WHAT DISTINGUISHES LEVEL 2 FROM OTHER LEVELS

VS LEVEL 1:

- Level 2 has 3-5+ failed attempts (Level 1: 1-3)
- Level 2 child is OFTEN mixed verbal (Level 1: fully verbal)
- Level 2 mom has BCBA routine (Level 1: often no BCBA)
- Level 2 mom is more skeptical (Level 1: still has hope)
- Pre-K pressure GONE for Level 2 (school already adapted with IEP)

VS LEVEL 3:

- Level 2 still has "maybe trained" hope (Level 3: "maybe never")
- Level 2 child is significantly younger
- Level 2 mom has not resigned yet
- Level 2 child is verbal (Level 3 often non-verbal)
- Level 2 reacts to science (Level 3 reacts to dignity)

5. THE LEVEL 2 PARADOX

THE PARADOX:

Level 2 is the HARDEST mom to convert — because she has seen enough to be skeptical, but not enough to have given up.

She is HYPER-SKEPTICAL + HYPER-MOTIVATED simultaneously.

WHAT THAT MEANS FOR MARKETING:

- Marketing polish triggers skepticism (HYPER-SKEPTICAL)
- But: she is ready to invest if mechanism convinces (HYPER-MOTIVATED)
- She will read the mail critically
- But: she will read it to the end if the hook holds

THE LEVEL 2 MAIL FORMULA:

1. REAL symptom recognition (she tests immediately)
2. Mechanism specificity (she searches for "real")
3. Acknowledge BCBA/OT work (she respects her team)
4. Realistic timelines (she's heard enough lies)
5. 60-day guarantee (she needs risk removal)
6. Permission-not-to-buy (she reacts to respect)

IF ONE ELEMENT IS MISSING, CONVERSION BREAKS.

PART B: PSYCHOLOGY OF THE LEVEL 2 MOM

6. THE 4 PHASES FOR LEVEL 2

For Level 2 families, Phase 3 (METHOD CYCLING) is the LONGEST and MOST IMPORTANT phase. Most BrightKidCo subscribers in this segment are here.

PHASE 1 FOR LEVEL 2: SLOW REALIZATION

Child 2.5–3.5 years

WHAT HAPPENS:

- First diagnosis conversations, waitlists
- Sensory issues show up early
- Speech development delayed
- Pediatrician says "Don't worry about potty yet"

- Parents focus on speech, sensory, sleep

EMOTIONAL STATE:

- Diagnosis processing dominant
- Potty is NOT priority
- First grief over "neurotypical path"

WHAT SHE READS:

- "is my 3 year old autistic"
- "speech delay autism signs"
- "developmental milestones autism"

EMAIL IMPLICATION:

- Level 2 mom rarely comes to BrightKidCo in this phase
- If yes: pure education, no sale
- Mechanism preview without hard pitch

CONVERSION PROBABILITY IN PHASE 1: VERY LOW (~2%)

PHASE 2 FOR LEVEL 2: FIRST REAL ATTEMPT

Child 3.5–4.5 years

WHAT HAPPENS:

- Diagnosis is there or in process
- Tries standard method on recommendation
- Child completely overwhelmed, sensory meltdowns
- She stops after 24-48 hours
- BCBA or OT says: "Wait, you need a different approach"
- Waits, feels disoriented

EMOTIONAL STATE:

- Deep shock — "my kid is REALLY different"
- Begins to accept: autism-specific needed
- Grieves "easier path"
- Self-doubt + anger at "mainstream advice"

WHAT SHE READS:

- "potty training autistic toddler"
- "my autistic 4 year old still in diapers"
- "autism sensory issues toilet"
- "abandoned oh crap method autism"
- "autism potty training meltdowns"

MAIL TONALITY:

- Validation of the grief ("It's hard when standard advice doesn't fit")
- Mechanism explanation becomes central
- "There's a different approach" language
- No hope selling — realism first
- Sensory awareness in language

CONVERSION PROBABILITY IN PHASE 2: MEDIUM (~10-15%)

PHASE 3 FOR LEVEL 2: METHOD CYCLING (LONGEST PHASE)

Child 4-7 years

HERE THE LEVEL 2 MOM OFTEN SPENDS MULTIPLE YEARS.

THIS PHASE = MAIN FOCUS FOR LEVEL 2 MARKETING.

Most Level 2 subscribers are here.

WHAT HAPPENS:

- Tries ABA protocol (BCBA gives data sheets, hourly protocols)
- Tries visual schedules, social stories
- Tries sensory regulation (deep pressure, weighted blankets before toilet)
- Has OT appointments 1-2× per week
- Some wins, many regressions
- School enrollment becomes crisis (resolved via IEP)
- Child gets older, still in pull-ups size 6, 7, 8
- First GI issues develop (withholding)

EMOTIONAL STATE:

- Standard skepticism at new offers
- "I've tried everything" is pain, not statement
- Becomes cynical toward brands
- But: not yet cynical toward science
- Hyper-vigilant toward "miracle product" language
- Immediately recognizes marketing bullshit
- Accepts "autism mom" identity
- Loss of friends without special needs kids

WHAT SHE READS:

- "autism 5 year old still in pull-ups"
- "interoception autism potty training" (often!)
- "potty training school readiness autism"
- "kelly mahler interoception" (Phase 3 advanced)
- "non-verbal autism potty training" (even if child verbal, she searches broadly)
- "ABA potty training not working"
- "autism withholding bowel movement"
- Research-oriented searches

EMOTIONAL STATE AT MAIL OPEN:

- Default skepticism ON
- Searches actively for "real solution"
- Ready to read if hook sounds real
- Tolerates longer mails if content is deep

MAIL TONALITY:

- SCIENCE-FIRST (Kelly Mahler, SPARK, OT quotes)
- Mechanism specificity > brand promises
- Specific testimonials (Level 2, 5-7 years, BCBA context)
- 60-day guarantee as hope-risk absorber
- Realistic timelines (8 weeks to first real wins)
- NO hard sell, never
- Permission-not-to-buy energy
- BCBA-respectful language

OPENERS THAT WORK:

- "You've done BCBA. You've done OT. You've done social stories, sticker charts, scheduled sits. Nothing's fully working — and you're starting to wonder if you missed something fundamental."
- "Sensory regulation work helped with everything except bathroom anxiety. Here's why that might be."
- "Your kid goes for the teacher but not for you. That's not defiance. That's prompt-dependency."

CONVERSION PROBABILITY IN PHASE 3: MEDIUM (~8-15%)

CAUTION: Lower conversion rate than Level 1 Phase 2, because skepticism is much higher. But HIGHER average order size (often 5+5 bundle).

PHASE 4 FOR LEVEL 2: RESIGNATION FORK

Child 7+ years

For Level 2, Phase 4 occurs in ~40% of cases.

WHAT HAPPENS:

- Child is 7+ and still in pull-ups
- Has taken 2-year break from active training
- Plans renewed attempt (often before important school event)
- Has accepted: school with pull-up possible (IEP-supported)
- But: deep shame when others find out

EMOTIONAL STATE:

- Grieves normal childhood
- Anger alternates with acceptance
- Often has partner conflict
- Sibling comparison intensifies pain
- Last-resort mode but respectful

MAIL TONALITY:

- Very respectful
- "It's not too late" with real late-trainer stories
- Maureen Bennie's son 9.5 as anchor
- Longer timelines (8-12 weeks)
- Pull-up difficulties as trust bridge
- NO pressure, never guilt

CONVERSION PROBABILITY IN PHASE 4: MEDIUM (~6-10%)

PHASE SUMMARY LEVEL 2:

Phase Share of L2 Audience Conversion Rate			
----- ----- -----			
1	~5%	2%	
2	~25%	10-15%	
3	~55%	8-15%	
4	~15%	6-10%	

→ Phase 3 is the "fat middle" for Level 2. Email strategy should dominate here.

7. SYMPTOM CLUSTERS (6 CLUSTERS)

These 6 clusters are Level 2 specific. In every mail there should be 2-3 of these clusters as recognition anchors. Cluster A (Sensory) + B (Withholding) are the strongest triggers.

SYMPTOM CLUSTER A: SENSORY DEFENSIVENESS AROUND TOILET

EXACT FORMULATIONS FROM RS:

"He won't sit on the toilet — the seat feels weird."

"The flushing sound makes her cry."

"The bathroom light is too bright for him."

"He hates the cold seat in winter."

"She panics when the toilet flushes."

"The bathroom has terrible acoustics for him."

"Public bathrooms are impossible — we just don't go."

"She needs the same toilet at home, anywhere else triggers meltdowns."

WHAT IT MEANS:

The bathroom itself is sensory hell. Before toilet training can start, sensory tolerance must be built.

EMAIL IMPLICATION:

~80% of Level 2 moms recognize themselves. Very strong recognition hook.

SYMPTOM CLUSTER B: WITHHOLDING + GI ISSUES

EXACT FORMULATIONS FROM RS:

"He holds his poop until bedtime when the pull-up goes on."

"She refuses to go for hours."

"He has chronic constipation."

"She'll sit on the toilet for 30 minutes, get up, and immediately go in her pull-up."

"He's afraid to poop, will hold it for days."

"He's on Miralax daily for 2 years."

"She has overflow incontinence — soils without realizing."

"He's been hospitalized for impaction twice."

WHAT IT MEANS:

Withholding is not behavior — it's fear/sensory trigger.
GI complications are not periphery, but central.

EMAIL IMPLICATION:

Very emotional trigger. But: mails must not promise
"miracle cure" for withholding. Validate, but don't
oversell.

SYMPTOM CLUSTER C: DELAYED VERBAL SIGNALS

EXACT FORMULATIONS FROM RS:

"He says 'wet' after the accident, never before."

"She uses scripted speech, can't tell me her needs in real time."

"He echoes 'do you need to pee?' instead of answering."

"She can label body parts but not feelings."

"He has the words but can't connect them to internal

sensations."

"She uses AAC for everything except bathroom needs."

"He'll talk about peeing in past tense after the fact."

WHAT IT MEANS:

Verbal available, but not for internal signals.

Communication tools (AAC, social stories) help for external things, not for interoception needs.

EMAIL IMPLICATION:

Important for Level 2 mom: "verbal doesn't mean independent" hits her deeply.

SYMPTOM CLUSTER D: SCHEDULE DEPENDENCY WITHOUT INTERNAL SIGNAL

EXACT FORMULATIONS FROM RS:

"He'll go when I take him every 90 minutes, but never on his own."

"She's been on a timed schedule for 2 years — still no independence."

"He's prompt-dependent, the BCBA confirms it."

"She goes for the teacher but not for me."

"He's compliant when prompted, never initiates."

"After 30 hours of ABA, he sits — but doesn't go on his own."

WHAT IT MEANS:

The child is "trained to comply", not "trained to feel".

BCBA has perfected the behavioral component, but sensory component is missing.

EMAIL IMPLICATION:

THAT is the most resonant frustration for Level 2 moms. They know prompt-dependency intimately.

SYMPTOM CLUSTER E: REGRESSION CYCLES

EXACT FORMULATIONS FROM RS:

"We had 3 weeks of success then total regression."

"She trained at school then forgot it all over summer break."

"He goes through phases — months of accidents, then weeks of dryness."

"Every change in routine sets us back to zero."

"Vacation always means starting over."

"COVID set us back 2 years of progress."

"Every illness erases what we built."

WHAT IT MEANS:

Progress is not linear, often frustratingly cyclical.

Routine sensitivity makes stability very difficult.

EMAIL IMPLICATION:

Mail language must acknowledge: regressions are not failure, are sensory recalibration.

SYMPTOM CLUSTER F: HOMETIE LOCATION-SPECIFIC TRAINING

EXACT FORMULATIONS FROM RS:

"He goes at home but never in public."

"She goes at school but not at home."

"He has a specific toilet he'll use — anywhere else, nothing."

"She's trained at grandparents' but not at our house."

"Travel is impossible — different bathroom = no go."

WHAT IT MEANS:

Generalization is missing. Child has local skills, but can't generalize them. Sensory triggers differ per location.

EMAIL IMPLICATION:

Medium strength, but very Level 2 specific.

USE CASE FOR THE 6 CLUSTERS:

In EVERY Level 2 mail, 2-3 of these clusters should appear as recognition anchors. Example combinations:

Welcome E2: Cluster A + Cluster D

Welcome E4: Cluster D + Cluster E

Cart Aband E1: Cluster B + Cluster D

FAQ E1: Cluster A + Cluster C

Browse Abandonment: Cluster D + Cluster E

8. VERBATIM LANGUAGE BANK — LEVEL 2 SPECIFIC

These quotes come directly from Level 2 family sources.

CATEGORY 1: HOW LEVEL 2 MOMS DESCRIBE THEIR CHILD

"He's 6 and we're still on a schedule. He hasn't initiated once."

"She knows what to do but the bathroom is too overwhelming."

"He holds his bowel movement until the pull-up goes on at bedtime."

"My daughter screams when the toilet flushes — we have to flush after she leaves the bathroom."

"He's so smart, but he can't tell me when he needs to go."

"She's been in pull-ups her entire life. She's 6 now."

"He goes for the teacher but won't even try at home."

"His BCBA says he's prompt-dependent and we need to fade prompts, but he just goes back to pull-ups when we do."

"I've tried everything from the BCBA's protocol to OT sensory work to social stories. We're stuck."

"He has chronic constipation. Miralax daily for 2 years."

"She uses an AAC device for everything but bathroom needs."

"He's been outgrowing pull-ups — we're moving to specialty diapers."

CATEGORY 2: HOW LEVEL 2 MOMS DESCRIBE FAILED METHODS

"We've been doing scheduled sits for 2 years. He never goes on his own."

"BCBA protocol got us to 'sit on the toilet without melting down.' That's it. No actual training."

"OT sensory work helped with other things, but not the bathroom."

"Social stories made her more confused, not less."

"We did a 'no pull-up' day. He held it for 11 hours then peed everywhere."

"I tried bribery, rewards, sticker charts, social stories, ABA protocols, scheduled sits, weighted blankets, special seats. Nothing changes the fundamental issue."

"Naked therapy at the autism center — meltdowns daily, no progress."

"We hired a private potty consultant for \$2000. Same result."

CATEGORY 3: HOW LEVEL 2 MOMS EXPRESS SELF-DOUBT

"I should have started sooner."

"His sensory issues are too severe for any product."

"Maybe his autism is too much."

"I think we missed the window."

"I'm not good at the BCBA stuff."

"I can't even get my husband on board with the schedule — he gives pull-ups when I'm at work."

"Maybe I'm not implementing things correctly."

CATEGORY 4: HOW LEVEL 2 MOMS DESCRIBE PROGRESS (WIN MOMENTS)

"He paused. He felt something."

"She didn't melt down when I changed her in the bathroom."

"He went 4 hours without an accident."

"She let me sit her on the toilet without screaming."

"He said 'pee' BEFORE the accident, once."

"She finally pooped in the toilet after a year of withholding."

"He initiated. Once. After 4 years."

"She told me, with her AAC, 'need bathroom.' That's a first."

"He went 6 hours between accidents. New record."

CATEGORY 5: HOW LEVEL 2 MOMS EXPRESS BCBA/OT FRUSTRATION

"BCBA says we need to fade prompts but I don't know how."

"OT says interoception but didn't explain what to do."

"His ABA team has worked on this for years — no real

progress."

"They focus on compliance, not actual independence."

"\$50k of ABA later, still in pull-ups."

"Every therapist suggests the next therapy."

CATEGORY 6: WHAT LEVEL 2 MOMS GOOGLE

"autism level 2 potty training boys"

"BCBA potty training not working"

"my autistic 6 year old won't initiate"

"sensory issues bathroom autism"

"withholding poop autism level 2"

"prompt dependent toilet training autism"

"interoception autism potty 6 year old"

"autistic child holds bowel movement"

"toilet training autism school but not home"

9. CONVERSION TRIGGER MAP FOR LEVEL 2

When exactly does the Level 2 mom decide to buy?

PRIMARY TRIGGER #1: PROMPT-DEPENDENCY FRUSTRATION

What she thinks: "He sits but doesn't go. After 2 years of ABA. We're stuck in compliance, not training."

Mail lever: "Prompt-dependency isn't training. Here's what the missing piece is."

Investment readiness: High

Timeline tolerance: 8-12 weeks

PRIMARY TRIGGER #2: NEW WITHHOLDING CRISIS

What she thinks: "He's been holding bowel movements for 5 days. We're heading to the hospital again. Something has to change."

Mail lever: Sensory regulation + sustained signal = key

Investment readiness: Very high

Timeline tolerance: variable

PRIMARY TRIGGER #3: SCHOOL CONTEXT CHANGE

What she thinks: "He transitions to elementary school in fall. His IEP includes pull-up support but other kids notice now."

Mail lever: "Add alongside IEP support. Build independence piece by piece."

Investment readiness: High

Timeline tolerance: 3-6 months

PRIMARY TRIGGER #4: COMPARISON MOMENT

What she thinks: "Younger sibling is trained. Cousins were trained at 3. He's 6."

Mail lever: Validation + Mechanism + Late-Trainer stories

Investment readiness: Medium-High

Timeline tolerance: variable

PRIMARY TRIGGER #5: OT RECOMMENDATION TO INTEROCEPTION

What she thinks: "OT mentioned interoception 6 months ago. I read Kelly Mahler. Now I'm looking for a product that addresses it specifically."

Mail lever: Direct Kelly Mahler reference, mechanism specificity

Investment readiness: Very high (clinically validated)

Timeline tolerance: long

WHAT SHE DOES AFTER TRIGGER:

1. Google search (see Category 6)
2. Reads posts in autism communities

3. OT/BCBA consultation
4. Insta research on autism-specific products
5. Clicks Meta ad or organic post
6. Lands on BrightKidCo
7. Pop-up: "Sensory issues, tried many methods" → click
8. Welcome flow starts
9. 7-21 days decision period (longer than Level 1!)
10. Purchase or drop-off

EMAIL IMPLICATION:

- Welcome flow designed longer for Level 2 (more decision time)
- Science prominent from start
- BCBA-respectful throughout
- No Pre-K pressure (often already irrelevant)

PART C: TESTIMONIAL & PROOF STRATEGY

10. TESTIMONIAL PATTERNS FOR LEVEL 2

CHILD PROFILE IN TESTIMONIAL:

- Age 5-7 years (sweet spot 6)
- "Diagnosed Level 2" or "moderate support needs"
- Partly verbal, sensory dominant
- 3-5 failed attempts mentioned
- BCBA / OT context in background prominent
- Optional: comorbidity (GI, anxiety)

TIMELINE IN TESTIMONIAL:

- 4-8 weeks to first mini-wins
- 3-6 months to daytime independence
- Acknowledged: night training comes MUCH later
- Realistic: "Still have accidents, but..."
- Regression cycles mentioned + acknowledged

LANGUAGE PATTERNS:

- "After 3 years of trying everything..."
- "His BCBA was skeptical but..."
- "OT recommended sensory regulation, this seemed to be the missing piece"
- "Week 6 was when it clicked"
- "Still working on poop, but pee is mostly there"
- "Some regression but overall trending up"

DETAILS THAT BUILD TRUST:

- BCBA / OT mentioned with name or specific years
- Specific therapy hours ("20 hours of ABA")
- Comorbidities mentioned (Miralax, anxiety, sensory)
- Authentic struggle moment
- Honest "still not done" closing

11. SAMPLE TESTIMONIALS

SHORT TESTIMONIAL (for mail inserts, ~4-6 sentences):

"My daughter is 6, Level 2, mostly non-verbal. We'd been trying for 3 years with BCBA, OT, social stories — she'd sit when prompted but never initiate.

After 5 weeks with the 3+3 bundle, she started PULLING at the front of the underwear right before an accident. I cried. Her BCBA noticed too — said she'd never seen this shift before.

We're 4 months in. Not 'fully trained.' But the signal is there where it wasn't before."

— Lisa, mom of [name], 6yo Level 2

MEDIUM TESTIMONIAL (for Welcome Education, ~10-15 sentences):

"Ethan is 6, Level 2, has been in ABA since age 3 — 20 hours a week. Sensory processing differences are his major challenge. His BCBA, his OT, his speech therapist — they've all worked on toileting. He's been on a 90-minute scheduled sit protocol for 18 months.

He'd sit. He wouldn't go. Then he'd wait until I changed him into a pull-up, and within 5 minutes, he'd pee. Classic prompt-dependency, his BCBA confirmed.

We have GI complications — chronic withholding, daily Miralax. The bathroom flushing makes him cover his ears.

I bought the 3+3 bundle in October — kind of as a 'why not' purchase. I was skeptical. I expected it to fail.

Week 1: more accidents. We had a meltdown about wet underwear. Hard.

Week 4: he paused. He felt the wetness. He looked at me. First time in 6 years.

Week 8: he initiates maybe 30% of the time at home. School is still BCBA-prompted, but home is different.

I've reduced his Miralax dose. His GI is improving.

Still not 'trained' in the way other kids are. But for the first time in years, his body is sending him a signal his brain can register. That's what was missing."

— Lisa, mom of Ethan, 6yo Level 2, 3 years of ABA

LONG TESTIMONIAL (for PDP, FAQ mails, 20+ sentences):

If marketer needs a longer testimonial, here is structure:

PARAGRAPH 1: Child profile + diagnosis + therapy history
PARAGRAPH 2: Specific symptoms (Sensory + Prompt-Dep + GI)
PARAGRAPH 3: Methods tried (detail!) + why failed
PARAGRAPH 4: Skepticism at purchase + expectations
PARAGRAPH 5: Week-by-week progress with concrete wins
PARAGRAPH 6: Where they are now + honest "still working on"
PARAGRAPH 7: Recommendation (peer-to-peer, not marketing)

PROOF RANKING FOR LEVEL 2:

#1 STRONGEST PROOF: BCBA-VALIDATING TESTIMONIAL

"Her BCBA had been working on this for 4 years. She noticed the shift in Week 5."

Why: She respects BCBA. When BCBA validates, she accepts the reality.

#2 SECOND STRONGEST: KELLY MAHLER CITATION + MECHANISM

"Kelly Mahler states: 'Rewards can't create or clarify internal body signals.' The Body-Signal Learning Layer addresses what behavior-based approaches cannot."

Why: She often already knows Mahler. Direct quote = authority.

#3 THIRD STRONGEST: SPECIFIC LONG-TERM TESTIMONIAL

"6-year-old, Level 2, 3 years of ABA, withholding history. Week 8 = first independent initiation."

Why: She's searching for kids LIKE MINE. Specificity builds trust.

#4 FOURTH STRONGEST: SPARK STAT CONTEXTUALIZED

"49% of autistic 4-5 year olds aren't trained — even with ABA, social stories, and therapy."

Why: Validates her experience. Adds clinical weight.

#5 FIFTH STRONGEST: 16,511 REVIEWS / 100,000+ PARENTS

Why: Trust signal but secondary. She has bought other "popular" products that didn't work. Volume less powerful than specificity.

PROOF PLACEMENT IN MAILS:

Welcome E1: reviews stat in footer, no body proof
Welcome E2: Kelly Mahler quote + mechanism
Welcome E3: 1 medium testimonial
Welcome E4: Kelly Mahler deepened
Welcome E5-6: Full testimonials
Cart Recovery: 1 specific Level 2 testimonial with BCBA
FAQ mails: Multiple testimonials depending on question
Post-purchase: Mini-wins pattern from testimonials

PART D: TONALITY & CALIBRATION

13. TONALITY CALIBRATION PER ELEMENT

HOPE LEVEL: LOWER than Level 1, HIGHER than Level 3

- No "this will work" language
- "This may close a gap" works
- Acknowledge: progress will not be linear
- Realistic, but not hopeless

REALISM LEVEL: HIGH

- "Some kids take 8 weeks. Others 3 months."
- Acknowledge regression cycles as normal
- "Not every kid responds. 60 days to know."

URGENCY LEVEL: LOW

- School is usually already settled (IEP)
- Pre-K pressure less acute
- "Before next school year" can be mentioned, but gently

SCIENCE DENSITY: HIGH

- Kelly Mahler quote prominent
- SPARK stat as trust anchor
- Interoception language OK (she often already knows)
- BCBA-respectful language (not against ABA, but complementary)

- Reference OT concepts

EMPATHY INTENSITY: HIGH

- She is tired and hurt
- "I see how exhausting this has been" works
- Validate specific situations (sensory meltdowns, withholding, scheduled sits frustration)
- Deep observation > shallow sympathy

TESTIMONIAL LENGTH: LONG

- Level 2 mom reads in more detail
- 10-15 sentences per testimonial ideal
- Mention multiple wins, not just one
- Realistic "not finished yet" language important

CTA AGGRESSIVENESS: LOW

- "See options when you're ready"
- "60 days to decide"
- NO urgency tricks
- Permission-not-to-buy strongly prominent

GENERAL ENERGY:

- Patient, deliberate
- Like an experienced OT who has seen many families
- Like a respectful counselor
- Not too warm (feels fake), not too clinical

14. HOPE VS REALISM BALANCE

THE LEVEL 2 BALANCE:

40% HOPE / 60% REALISM

Hope language (sparingly):

- "This may close one piece of what's been missing"
- "Some families see real change after 4-8 weeks"
- "Worth trying given everything you've already done"

Realism language (dominant):

- "Some kids take 8-12 weeks. Some longer."

- "Regression cycles are normal — that's not failure"
- "60 days to see if this fits your kid"
- "Not every kid responds — and you'll know"
- "Mini-wins count: pause, look, pull at pants"

WHY 40/60?

Level 2 is wounded by too much hope selling. She needs more realism than Level 1 — otherwise it sounds like all other brands.

But: completely hope-free = "why try then?"

Balance is: enough realism to build trust, enough hope to motivate decision.

15. BCBA RESPECT LANGUAGE

CRITICAL FOR LEVEL 2.

She often has 4-6 years of BCBA relationship. If mail sounds like it criticizes BCBA, she's gone immediately.

DO LANGUAGE:

- ✓ "Your BCBA has been doing the hard work for years"
- ✓ "ABA addresses behavior. The sensory piece needs different input."
- ✓ "Many BCBAs incorporate sensation-based feedback into protocols once they see results"
- ✓ "This complements your ABA program, not replaces it"
- ✓ "Discuss with your BCBA — bring data after 30 days"

DON'T LANGUAGE:

- ✗ "ABA doesn't work for potty training"
- ✗ "Your BCBA missed this"
- ✗ "BCBA protocols are flawed"
- ✗ "Stop the ABA protocol and try this"

KEY MESSAGE:

"ABA + Body-Signal = both pieces"

PART E: CONVERSION LEVERS FOR LEVEL 2

16. HOW THE BIG-5 LEVERS WORK FOR LEVEL 2

LEVER 1 ("It's not the method"): VERY HIGH

Effect for Level 2:

- She has MASSIVE method cycling behind her
- "I've tried everything" is her reality
- Reframe works very strongly
- Named methods + why failed = resonance

Use:

- Welcome E2 prominently
- Welcome E4 expanded
- FAQ E1 central
- Cart Recovery E2

LEVER 2 ("Pull-ups make this worse"): HIGH

Effect for Level 2:

- She has used pull-ups for years (often 4-6 years)
- Math reframe works (\$876/year)
- But: pull-ups are safety net
- Language must be: "Add alongside" not "abandon"

Use:

- Welcome E2 (Mechanism)
- Welcome E5
- Browse Abandonment
- Cart Recovery
- Caution: not too aggressive vs. pull-ups

LEVER 3 ("60-Day Guarantee"): VERY HIGH

Effect for Level 2:

- Skepticism high, risk reduction critical
- "Try without commitment" is trust maker
- She has lost \$500-2000 in other products
- 60-day timeframe fits regression cycle reality

Use:

- Welcome E3 (Last Day)
- Cart Recovery final
- Checkout Abandonment
- FAQ mails
- Every conversion-oriented mail

LEVER 4 ("Lena's two autistic sons"): VERY HIGH

Effect for Level 2:

- Peer-to-peer voice is gold
- Lena's sons are Level 2-like (experience with long-term training)
- Trust through similar experience
- "She's been where I am" effect very strong
- 8-year-training reality validates her timeline

Use:

- Lena sign-off in EVERY mail
- PP-Empathy moments
- Reply encouragement
- Welcome E1+E2 Lena prominent

LEVER 5 ("49% SPARK Stat"): VERY HIGH

Effect for Level 2:

- Validates her experience massively
- "I'm not alone, this is real" effect
- Combined with "even kids in BCBA programs"
- Clinically validated

Use:

- Welcome E1+E2
- FAQ E1
- Winback

17. LEVEL 2-SPECIFIC ADDITIONAL LEVERS

ADDITIONAL LEVER L2-A: "COMPLEMENTS, DOESN'T REPLACE BCBA/OT"

Language:

"Your BCBA protocol works on behavior. OT works on regulation. The signal gap — interoception — neither traditionally addresses. This adds that piece."

Effect:

- Very strong, because she respects BCBA/OT but feels it's not enough
- Reduces fear of "going against the BCBA"

Use:

- Welcome E2 + E4
- Cart Recovery E1
- FAQ on BCBA compatibility

ADDITIONAL LEVER L2-B: "BREAK THE REGRESSION CYCLE"

Language:

"Most Level 2 families describe progress-regression cycles. That's not failure — it's because the sensory signal never became reliable. Adding sustained wetness sensation can stabilize the cycle."

Effect:

- Hits exactly her reality
- Names specific frustration
- Solution frame without miracle claim

Use:

- Welcome E4
- Winback
- Cart Recovery E2

ADDITIONAL LEVER L2-C: "BCBA-COMPATIBLE"

Language:

"This works alongside scheduled sits and visual schedules. Many BCBAs incorporate sensation-based feedback into protocols once they see results."

Effect:

- Reduces fear of "doing it wrong"
- Creates "talking point" with BCBA

Use:

- Cart Recovery
- FAQ
- PP-Education

ADDITIONAL LEVER L2-D: "BEYOND PROMPT-DEPENDENCY"

Language:

"Prompt-dependent kids comply when reminded but never independently initiate. That requires building the internal signal, not just the routine. Here's how."

Effect:

- Names specific frustration she has daily
- Very resonant for 70%+ of Level 2 moms

Use:

- Welcome E2 or E4
- Cart Recovery
- FAQ E1

ADDITIONAL LEVER L2-E: "WITHHOLDING AS SENSORY ISSUE"

Language:

"Withholding is often interpreted as defiance. It's almost always sensory: fear of the toilet, fear of the sensation, or inability to recognize the urge until it's overwhelming. Body-signal training can ease this slowly."

Effect:

- Hits 50%+ of Level 2 moms directly
- Very emotional, very resonant
- But: CAUTION, no withholding cure promise

Use:

- FAQ Withholding mail

- Cart Recovery (subtle)
- Welcome E5 (Education)

18. SENSORY REGULATION AS MASTER DRIVER

For Level 2, sensory regulation is the dominant driver.

HOW TO ADDRESS SENSORY:

DO:

- ✓ "Designed for hyper- and hypo-responsive sensory systems"
- ✓ "Sustained wetness sensation for kids who need more signal"
- ✓ "Sensory regulation work + body signal = both pieces"
- ✓ "For kids whose nervous system needs different input"

DON'T:

- ✗ "Cure sensory issues" (no one can)
- ✗ "Reduce sensory sensitivity" (not what we do)
- ✗ "Help with sensory meltdowns" (overpromise)

SENSORY-SPECIFIC CONVERSION MOMENTS:

- Mom battling bathroom meltdowns → Lever A (sensory acclimation)
- Mom has had OT consultation → Lever A + B (interoception lens)
- Mom has ABA still-sit routine → Lever C+D (prompt-dep)
- Mom has withholding crisis → Lever E (carefully)

PART F: STRUCTURAL GUIDELINES

19. FORBIDDEN PHRASES — LEVEL 2 SPECIFIC

In addition to DOC 1 Forbidden Phrases:

- ✗ "Easy" / "Quick" in any form
→ She knows it's not easy
- ✗ Direct Pre-K urgency
→ School is usually already adapted, not acute anymore
- ✗ "Just like other kids"
→ She has accepted: her child is different, not "just like"
- ✗ Aggressive BCBA criticism (see Section 15)
→ She respects her BCBA, wants no conflict
- ✗ "We can train your child in X weeks"
→ Immediate skepticism
- ✗ "Don't give up!" cheerleader language
→ Feels intrusive, she needs science not motivation
- ✗ Comparison with Level 1 stories
→ Frustrates her ("that's not my kid")
- ✗ School as main pressure
→ School is not her acute pain
- ✗ "Years of regret" or similar
→ Hits Phase 4, not Phase 3
- ✗ "Magic" / "Miracle"
→ Instant skepticism
- ✗ "Just trust the process"
→ Feels patronizing

20. SAMPLE OPENERS PER FLOW (LEVEL 2)

WELCOME E1 (Day 0 — Code Delivery):

Option A (Skeptical Audience):

"Your code's below. I get that the code might be the least interesting thing in this email — especially after years of trying. The rest of the email is for when you're ready to look closer at why nothing's worked."

Option B (BCBA Audience):

"Your code's below. If you've done BCBA, OT, social stories — you've heard 'wait until they're ready' and 'just need more time.' Here's a different framework worth a look."

WELCOME E2 (Day 1 — Mechanism):

"You've done BCBA. You've done OT. You've done social stories, sticker charts, scheduled sits. Nothing's fully working — and you're starting to wonder if you missed something fundamental. You haven't missed anything. The piece nobody mentions is interoception."

WELCOME E3 (Day 3 — Last Day):

"Today's the last day for the code. If you're still deciding, here's the question most Level 2 parents ask me: 'Will it work alongside our BCBA program?' Yes — and here's how."

WELCOME E4 (Day 7 — Education):

"You didn't fail at Oh Crap. The method assumed something your kid couldn't quite do yet — and here's exactly what. Plus: why prompt-dependency isn't training."

WELCOME E5 (Day 10 — Mechanism Deep Dive):

"Pull-ups remove the wetness signal. That's the mechanism nobody addresses. Here's what we built instead — designed for hypo-responsive interoception systems."

CART ABANDONMENT E1 (Hour 1):

"You looked at the 3+3 bundle yesterday. You're probably

wondering if anything can work at this point — after everything you've tried. I want to answer the question you might be sitting with: what makes this different than the last 8 things?"

CART ABANDONMENT E2 (Day 1):

"Most Level 2 families describe progress-regression cycles. The Body-Signal Learning Layer doesn't eliminate them — but it stabilizes the underlying signal. Here's what that means in practice."

BROWSE ABANDONMENT E1 (Day 1):

"You read about the body-signal layer yesterday. The piece most parents come back to re-read is the part about why prompt-dependent kids don't translate to independent initiation. That's not behavior — that's neurology."

CHECKOUT ABANDONMENT E1 (Hour 1):

"Your checkout's still open. If it's the 'will it work alongside our BCBA' question — yes, it's designed to complement, not replace. 60-day guarantee covers you either way."

POST-PURCHASE LEVEL-DETECTION (Day 14):

"You've had the underwear for about a week. Before I send the next round of support, can I ask one question about your kid? It helps me send the right kind of advice."

WINBACK PATH A:

"It's been a while. I'm not chasing — but if your kid is still in the prompt-dependent loop after years of ABA, here's a small experiment worth re-considering."

21. SUBJECT LINE PATTERNS FOR LEVEL 2

WELCOME FLOW SUBJECTS:

E1:

- "After 3 years of trying everything..."
- "The piece OT mentioned but didn't explain"
- "Your code + the framework BCBA misses"

E2:

- "BCBA protocol isn't enough. Here's why."
- "Sensory + signal: two different gaps"
- "What 'interoception' actually means for potty"

E3:

- "Last day for the code"
- "The BCBA-compatible piece you've been missing"
- "Last day + the regression cycle question"

E4:

- "Why she goes for the teacher, not for you"
- "Prompt-dependency isn't training"
- "Why withholding isn't defiance"

E5:

- "Pull-ups vs body-signal: the actual difference"
- "Designed for hypo-responsive nervous systems"
- "What 'sustained wetness sensation' means"

CART ABANDONMENT SUBJECTS:

E1:

- "Your cart's still here"
- "The 'will it work with our BCBA' question"
- "Why this is different than the last 8 things"

E2:

- "Stabilizing the regression cycle"
- "Most Level 2 families see week 4-6"
- "Working alongside what's already working"

E3:

- "Final reminder — your cart"
- "60-day guarantee covers you"
- "The math + the regression cycle"

BROWSE ABANDONMENT SUBJECTS:

E1:

- "You read about prompt-dependency"
- "The part most parents re-read"
- "Why behavior-training plateaus"

POST-PURCHASE SUBJECTS:

Day 0 (Order Confirmation):

- "Your order is in"
- "Realistic Week 1 expectations"

Day 14 (Level Detection):

- "Quick question about your kid"
- "Help me help you — 30 seconds"
- "To send the right kind of support, can I ask..."

SUBJECT LINE NO-GOS FOR LEVEL 2:

- ✗ "Pre-K is coming" (too Level 1)
- ✗ "He just needs more time" (trigger)
- ✗ "It's never too late!" (marketing tonality, bad)
- ✗ "Quick training tips" (too light)
- ✗ "Easy potty training" (instant skepticism)
- ✗ "Miracle for autism" (instant block)

22. BUYER JOURNEY MAPPING FOR LEVEL 2

AWARENESS (Pre-Subscribe):

- Has research-oriented search behavior
- Reads science articles on interoception
- Follows Kelly Mahler or autism researchers
- Clicks mechanism-oriented ads
- Pop-up: "Sensory issues, tried many methods" → click
- Highly considered email opt-in

WELCOME E1 (Day 0):

- Opens skeptically
- 30-60 seconds attention

- Immediately tests language for authenticity
- Click on code: 15-25% (lower than Level 1)
- Purchase in this mail: 2-5% (very low)

WELCOME E2 (Day 1):

- CRITICAL — decides if she stays
- Mechanism specificity is evaluated here
- 2-3 min attention if mechanism convinces
- Tipping point: "interoception" + BCBA-respectful
- Click: 20-25%
- Purchase: 4-8%

WELCOME E3 (Day 3 — Last Day):

- Decision time
- 60-day guarantee + permission-not-to-buy decisive
- Purchase: 5-10%

WELCOME E4-E8 (Day 7-21):

- Continued education
- Conversion steady between 2-6% per mail
- Long-tail conversions common in Welcome E5-E8

CART ABANDONMENT JOURNEY:

- Has cart laid down after long research
- Doubt is high
- E1 (1h): high open (65-75%), moderate recovery (10-15%)
- E2 (24h): deeper push with mechanism (8-12%)
- E3 (72h): final push with guarantee (5-8%)

POST-PURCHASE JOURNEY:

Day 0 (Order Confirmation):

- Relief + buyer's remorse
- Reassurance critical
- Cross-sell spot open (Wash Bag, Mattress)

Day 7-10 (Shipping/Arrived):

- Anticipation mixed with skepticism
- Open rate: 65-75%
- Upsell moment for companion products

Day 14 (Level Detection):

- Has used 1 week
- Reply rate critical for tag migration

- High engagement when opened

Day 30+ (Education):

- Either retention or refund consideration
- Mini-win acknowledgment critical for retention
- Regression cycle validation helps enormously

PART G: VISUAL & DESIGN CALIBRATION

23. VISUAL CUES FOR LEVEL 2 MAILS

HERO IMAGES:

✓ Real-life context:

- Kid (5-7) in calm setting
- Mom-kid bonding moment (subtle)
- Therapy context possible (OT setting, sensory tools)
- Home bathroom setting OK
- No action, more reflective

✓ Sensory-aware imagery:

- Soft textures
- Muted lighting
- Calm color palette
- Sensory tools visible (weighted blanket, fidget)

✗ Avoid:

- Heavy struggle moments
- Diaper change scenes
- Stock family happiness
- Pre-K classroom contexts (too Level 1)
- Older kids (8+) — too Level 3

COLOR ACCENTS:

Brand-Green dominant

Mint for highlight boxes (mechanism explanations)
Cream warmth (muted, not too bright)
Soft greys for secondary text

DESIGN PATTERNS:

- LONGER sections allowed (she reads deeper)
- More whitespace (sensory-friendly)
- Science boxes (highlight pattern for quotes)
- "Section breaks" with subtle dividers
- 2-column compare blocks for Pull-up vs BSL

24. IMAGE DIRECTION FOR LEVEL 2 MAILS

IMAGE DO'S:

- ✓ 5-7 year old children
- ✓ Calm settings (home, OT practice possible)
- ✓ Sensory-aware imagery (weighted blankets, fidget tools visible)
- ✓ Mom as reflective caregiver (not overwhelmed)
- ✓ Diverse representation
- ✓ Soft natural lighting
- ✓ Subtle "therapy-savvy parent" cues (notebooks, charts in background)

IMAGE DON'TS:

- ✗ Hyperactive happy-family stock
- ✗ Pre-K classroom settings (misses)
- ✗ Toddler-pure (misses)
- ✗ Non-verbal kid focus (misses — Level 3)
- ✗ Crisis moments (too heavy)
- ✗ Bright cartoon palettes
- ✗ Adult-diaper imagery

PRODUCT IMAGES:

- Underwear in size for 5-7 years
- Clear, clean, without dramatic context

- Sensory feel visualized (texture close-up OK)
- Body-Signal Layer diagram (technical, clinical)

25. TONE MARKERS IN DESIGN

DESIGN TONE FOR LEVEL 2:

CLINICAL WARMTH

- Science boxes (mint with quote)
- Mom-voice quotes (cream card)
- Mechanism diagrams (visual but tech-aware)
- Plenty of breathing room

NOT:

- Overly warm (feels fake)
- Clinically cold (loses empathy)
- Marketing polish (instant skepticism)
- Cartoon kid look

PART H: SPECIAL LEVEL 2 DYNAMICS

26. BCBA LOYALTY

Level 2 moms often have years-long relationship with BCBA.
Mails must:

DO:

- Position BCBA as partner
- Don't conflict with BCBA methods
- "Complement, not replace" framing throughout
- Speak BCBA as "respected ally"
- Suggest: "Discuss with your BCBA, bring data"

DON'T:

- Criticize BCBA (not even subtly)
- Use anti-ABA language
- Imply: "BCBA did it wrong"
- Say: "You must leave BCBA"

LANGUAGE PATTERNS:

"Your BCBA has built the behavioral foundation. This adds the sensory layer."

"Many BCBAs integrate sensation-based feedback once they see results."

"Bring 30 days of data to your next BCBA session — they'll recognize the patterns."

27. SCHOOL/IEP CONTEXT

Many Level 2 kids have IEP (Individualized Education Plan).
Mails can benefit from this:

DO:

- "Discuss with your IEP team"
- "Bring it up at the next IEP meeting"
- IEP as trust marker (you know what she means)
- Acknowledge school as supportive system

DON'T:

- Use school as pressure/urgency (that's Level 1)
- Criticize IEP limitations
- "School should accommodate this" demanding tone

LANGUAGE PATTERNS:

"Your IEP team already supports him in pull-ups. This just adds a piece you can build alongside."

"30 days of data = useful for the next IEP review."

28. COMORBIDITY REALITY

Many Level 2 children have ADHD, anxiety, GI issues, sleep problems. Mails can:

DO:

- "For kids with co-occurring ADHD or anxiety"
- "GI issues often complicate this — we'll address that"
- Address comorbidities as reality
- Acknowledge: "Withholding is often a sensory-fear loop"

DON'T:

- Ignore comorbidities (loses credibility)
- Promise cures for comorbidities
- "This will help everything" (overpromise)

LANGUAGE PATTERNS:

"For kids with chronic constipation, this is daytime-focused. Your medical team continues to manage the GI piece."

"Anxiety around the bathroom is common. Sustained signal helps the brain learn the bathroom is predictable, not scary."

29. PARTNER CONFLICT

More common for Level 2 than Level 1. Mails should:

DO:

- Not assume both parents are committed
- "Even if your partner uses pull-ups sometimes" acknowledge
- Acknowledge co-parenting reality
- Treat mom as primary decision-maker

DON'T:

- Require "discuss with your partner first"
- Imply: co-parent is undermining
- Use aggressive language against co-parent

LANGUAGE PATTERNS:

"Even if your co-parent is more cautious about transitioning out of pull-ups, daytime trials at home can start small."

"You don't need anyone's permission to try this. 60-day guarantee = no commitment."

30. SENSORY PROFILE COMPLEXITY

She knows sensory profiles (hyper/hypo-responsive). Mails can:

DO:

- "Hypo-responsive kids need MORE signal, not less"
- "For kids with sensory defensiveness around the bathroom"
- Sensory language shows expertise
- Hyper- and hypo-distinguishing

DON'T:

- "Just sensory issues" (too reductive)
- "Sensory" as buzzword without substance

LANGUAGE PATTERNS:

"For kids who are hypo-responsive (don't feel signals clearly), sustained wetness sensation closes that gap."

"Hyper-responsive kids may need slower transition — start with brief wear during play."

PART I: CHEAT SHEET & REFERENCE

31. LEVEL 2 MAIL QUICK REFERENCE

WHEN WRITING A LEVEL 2 MAIL, CHECK:

BEFORE WRITING:

- ☐ Which phase is the mom? (Phase 3 is most common)
- ☐ Which of the 6 symptom clusters fits?
- ☐ Did I include BCBA/OT respectfully?
- ☐ Which flow + which mail position?

WHILE WRITING:

- ☐ Did I mention 2-3 symptom anchors?
- ☐ Did I balance hope-realism 40/60?
- ☐ Did I position BCBA as partner?
- ☐ Did I use realistic timelines (8 weeks+)?
- ☐ Is science density high enough (Kelly Mahler, SPARK)?
- ☐ Lena sign-off per Section 20?
- ☐ Did I use empathy without cheerleader tonality?

AFTER WRITING:

- ☐ Forbidden phrases checked (Section 19)?
- ☐ Tonality calibration fits?
- ☐ Mobile-first layout (longer sections OK)?
- ☐ Subject lines 3 options created?
- ☐ Preheaders 3 options created?
- ☐ Did I avoid Pre-K (no Level 1 driver)?
- ☐ Did I avoid "miracle" or "easy"?

ONE-LINE SUMMARY OF LEVEL 2:

"The child has tried everything, is prompt-dependent, with sensory triggers, and the mom is tired but not yet resigned."

32. DELTA COMPARISON TO OTHER LEVELS

Dimension	Level 1	Level 2	Level 3
Child Age	3-5	5-7	6-10+
Verbal Status	Verbal	Mixed	Non/minimal
Tried Methods	1-3	3-5+	5+ (years)
Pull-up Size	4-5	5-7	7-8+, adult
Hope Level	Intact	Cracks	Accepted
Skepticism	Low	Very high	Deep, calm
Pre-K Pressure	Very strong	Weak	Not relevant
BCBA Context	Rare	Almost always	Long-term
GI Issues	Rare	Common (50%+)	Very common
Sensory Severity	Mild	Moderate	Severe
Hope Language	OK	Careful	Minimal
Empathy Intensity	Medium	High	Very high
Science Density	Low-Medium	High	Medium-high
Testimonial Len.	Short-Medium	Long	Long+messy
CTA Aggressive.	Medium	Low	Very low
Urgency Allowed	Some	Little	Never
Late-Trainer Stor	No	Sometimes	Central
Medicaid Sens.	Low	Medium	High

33. CROSS-REFERENCES

For cross-level mails:

→ DOC 5 (General Fallback) for cross-level language

For Cart/Checkout/Browse Abandonment:

→ DOC 5 dominant (cross-level)

→ If Level 2 tag known: DOC 3 for calibration

For Welcome Flow Level 2 variant:

→ DOC 3 (this doc) primary

→ DOC 1 for brand voice and master foundation

For FAQ/Objection (Phase 3):

→ DOC 3 for Level 2 variant

→ DOC 1 objection map section

For RS-PDF consultation:

→ Autistic_Parent.pdf

→ Autistic_language_Bank.pdf

→ Autistic_MIRO_m.pdf

END OF DOC 3 EN — LEVEL 2 PROFILE

NEXT RESPONSE: DOC 4 EN — LEVEL 3 PROFILE
